Yale JACKSON SCHOOL OF GLOBAL AFFAIRS

Adapting to Withdrawal from Development Assistance for Global Health

Midpoint Check-In

October 2026

Agenda

90 minutes

01	Introduction	5 min
02	OECD Aid Cut Projections	10 min
03	Dashboard Presentation	15 min
04	Findings	20 min
05	Looking Ahead	10 min
06	Discussion	30 min

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Objectives

01 Recap

02 Present New Findings

03 Re-align on Goals

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Recap

Key Takeaways from Kickoff Meeting

- ✓ Broadening scope beyond US MOUs to the full donor landscape (Gavi, the Global Fund, OECD bilateral aid)
- ✓ Shifting from “lives saved” output metric toward how services are actually affected, using a systems-level lens
- ✓ Examining how countries are responding, including their cofinancing commitments to different donors
- ✓ Organizing the data into a country-level dashboard, then using it to model coverage metrics

Executive Summary

Finding 1: Cuts were uneven relative to need and capacity

- Projected OECD cuts for 2026 averaged about 15% of last reported data, regardless of income or debt burden
- **Emerging Recommendation**: Aid reductions need to be tailored to each country's need and ability to adapt

Finding 2: Domestic revenue growth is unlikely to resolve shortfalls

- Reaching max tax potential covers the decline in 51 of 66 countries. The other 15 also need to raise health's share of the budget, which may prove to be difficult
- Governments may also have reduced domestic health financing in response to aid cuts, which would widen the gap.
- **Emerging Recommendation**: Aid recipients should pursue both tax revenue efficiency and budget reprioritization
- **Emerging Recommendation**: Development banks and other lenders concerned with health should offer concessional loans tied to health expenditure to bridge the gap

Finding 3: Foreign dependency makes some programs and countries especially exposed

- Programs like TB, malaria and HIV rely heavily on donor funds, and the US share of HIV treatment aid is 80% or more in 12 of 19 evaluated countries
- **Emerging Recommendation**: Remaining donor funds should target programs unlikely to be adopted by domestic spending

02

OECD Aid Cut Projections

Title

Blurb

How we built the projections

Projected aid = 2023 aid × a multiplier (0.8 means 80% of the money is left, a 20% cut)

1	**Baseline**	Split 2023 health aid ($27.6B, IHME) into ~20,500 pieces: one donor, one country, one channel.	**e.g.** “UK → Kenya via the Global Fund” is one piece.
2	**Announcements**	Where an announcement exists, it decides the multiplier: US deals, Global Fund, Gavi, UK and Australian budgets, named exits.	**e.g.** The US–Zambia health deal sets US aid to Zambia.
3	**Donor budgets**	In the absence of any announcement, the OECD forecast sets the donor’s total. The remaining is shared evenly over its other pieces.	**e.g.** Japan: $1.11B target, $0.30B announced → other pieces × 0.88.
4	**Add up**	Add the pieces up by country, donor, health area and year, 2026–2030, in constant 2023 dollars.	**e.g.** Kenya’s total = all its pieces from every donor.
5	**Ranges**	Rerun the model with low and high budgets, plus 2,000 simulated ways donors could split unannounced cuts based on historical data.	**e.g.** All countries, 2030: −25%, range −13% to −39%.

54% of the projected cut comes from step 2’s country-specific announcements. The rest comes is accounted for in step 3, where we know the donor’s total cut, but not which countries lose it.

Main findings: health aid falls about 25% by 2030

−25%

Decrease in health aid from all donors, 2023 to 2030

Most donors are cutting aid

US health aid falls 66%. Other governments fall 22%, led by Germany, France, UK, Netherlands.

Global health funds decrease

France cut its Global Fund pledge 58%, Japan cut 20%.

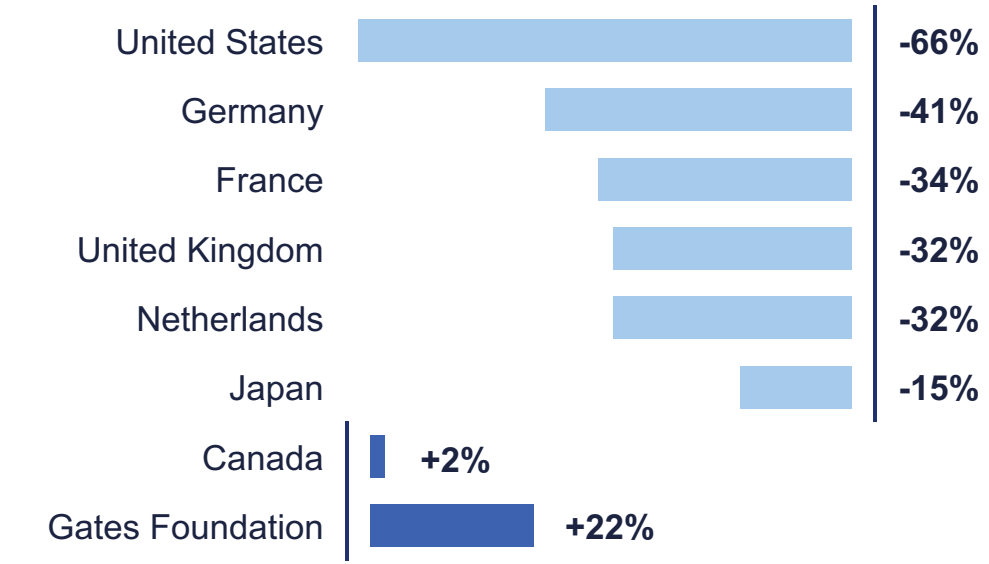
HIV, malaria and TB

Even without the US: malaria −30%, HIV −27%, TB −26%.

US gap

US health aid falls 66%, but only Canada and Gates grow.

Change by donor, 2030 vs 2023



Private philanthropy held flat (0%). Gates grows 2.9% a year by assumption.

Range by 2030: −13% to −39%. After inflation, health aid falls about 25% by 2030: around 13% with optimistic assumptions, 39% with pessimistic assumptions (eg: smaller donor budgets, less of the US deals actually delivered).

Source: our model on IHME 1990-2025 health aid, OECD aid outlook and donor announcements; constant 2023 US$.

Assumptions and caveats

Key assumptions

1. **US deals delivered as announced.** They are non-binding and need Congressional approval.
2. **Philanthropy:** Gates grows 2.9% a year (IHME), other private aid held flat. Together 32% of the baseline.
3. **US aid with no deal** kept at 25% of its old level; no country evidence exists [can change]
4. **Health keeps its share** of each donor's total aid as budgets shrink.
5. **Unannounced cuts spread evenly** within each donor: about a quarter of the cut.
6. **Judgment calls:** South Africa 0.1 and Zimbabwe 0.5 in 2026; exit year 0.5; Germany's wind-down.

Caveats

- **Cuts may come faster.** IHME estimates 2025 health aid already fell 67% (US) and 64% (UK).
- **Maternal and child health** cuts in US deal countries are probably understated.
- **2029–2030** rely on inputs held flat after 2028 (OECD outlook, UK, Dutch, Global Fund).
- **Country budgets = health budgets:** UK and Australian budgets cover all aid, and we assume health moves proportionally.
- **Gavi country allocations aren't public;** we only apply its stated plan to give fragile countries a bigger share.
- **Scope:** donor aid only. No recipient co-financing, no non-OECD donors.

Central assumptions: whether US deals are actually delivered, and if countries will cut aid uniformly between recipients

03
Dashboard

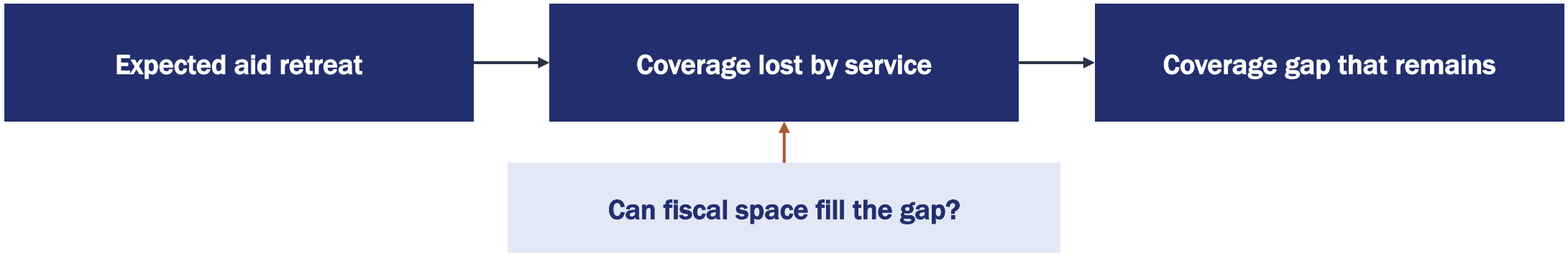
The Dashboard Links Aid, Budgets And Health Data To Show Coverage At Risk From Aid Cuts

26 DATASETS | 16 ORGANIZATIONS | 32 PUBLISHED STUDIES | 70 COUNTRIES

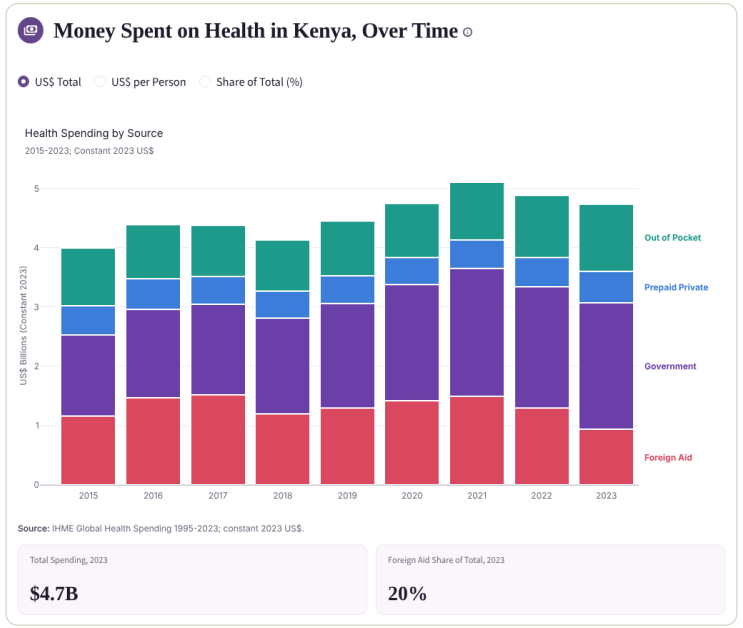
AIM 1: AGGREGATE DATA

FINANCIAL	**Government finances** IMF World Economic Outlook; World Bank WDI; IHME GDP	**Health spending by source** IHME Health Spending 1995-2050; WHO Global Health Expenditure Database	**Health aid by channel** IHME Development Assistance for Health 1990-2026; KFF MOU tracker; OECD 2025 aid; Gavi and Global Fund replenishments
HEALTH	**Need and current coverage** UNAIDS 2026; WHO TB, GHO and World Malaria Report; Malaria Atlas Project; WHO/UNICEF vaccine coverage; UN population	**Cost per person served** GHCC unit costs; Value TB; Rosen 2021; Optima HIV and WHO GHED spending; Global Fund budgets; published cost studies	**Effect of losing care** Optima HIV (ten Brink 2025); WHO TB; Lives Saved Tool; Vaccine Impact Modelling Consortium

AIM 2: MODEL HOW CUTS TO DEVELOPMENT AID AFFECT COVERAGE FOR EACH SERVICE



Dashboard – Domestic government and international aid contribute to health spending

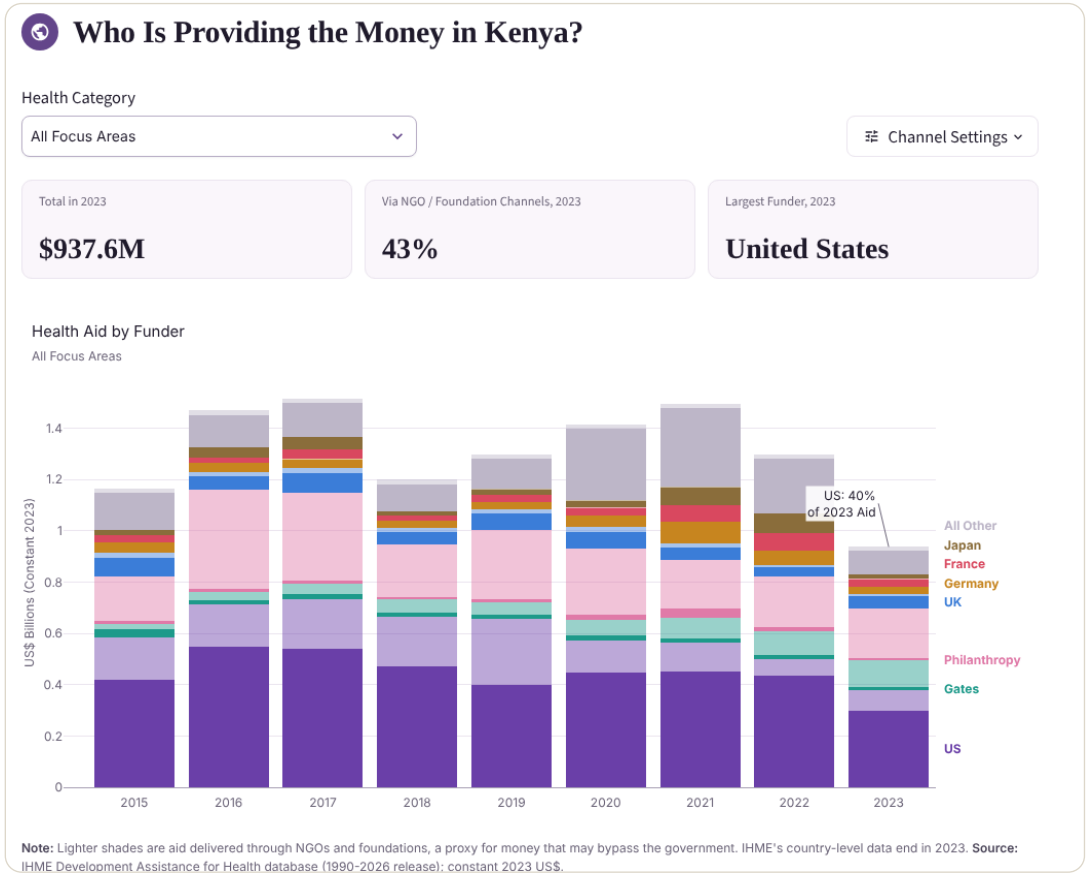


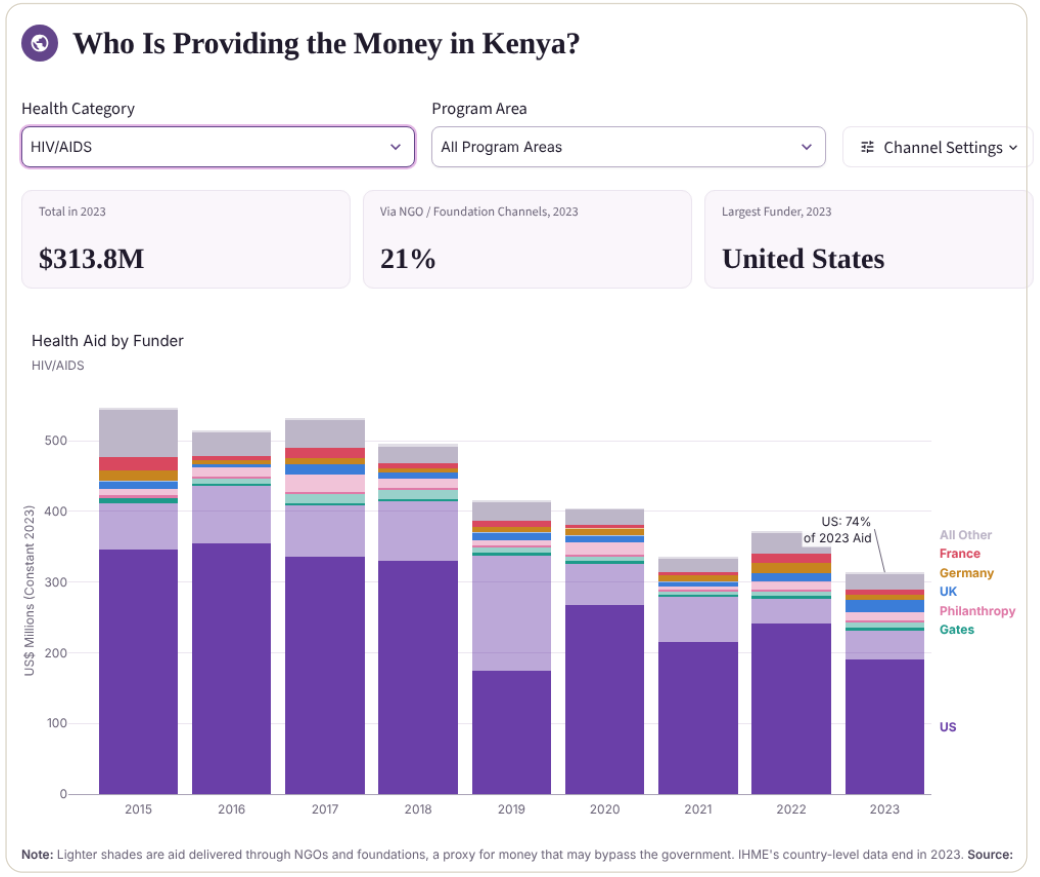


$$\mathrm{THE}_c = \mathrm{GHES}_c + \mathrm{DAH}_c + \mathrm{PPS}_c + \mathrm{OOP}_c$$
$$\phi_c = \mathrm{DAH}_c \,/\, \mathrm{THE}_c$$
$$G_c = R_c + B_c$$
$$h_c = \mathrm{GHES}_c \,/\, G_c$$

THE_c	total health expenditure in country c
GHES_c	government health spending
DAH_c	development assistance for health
PPS_c	prepaid private spending
OOP_c	out-of-pocket spending
ϕ_c	aid dependence
$G_c,\ R_c,\ B_c$	government spending, revenue, borrowing
h_c	health share of government spending

Dashboard – Resolving how development assistance for health funds different services





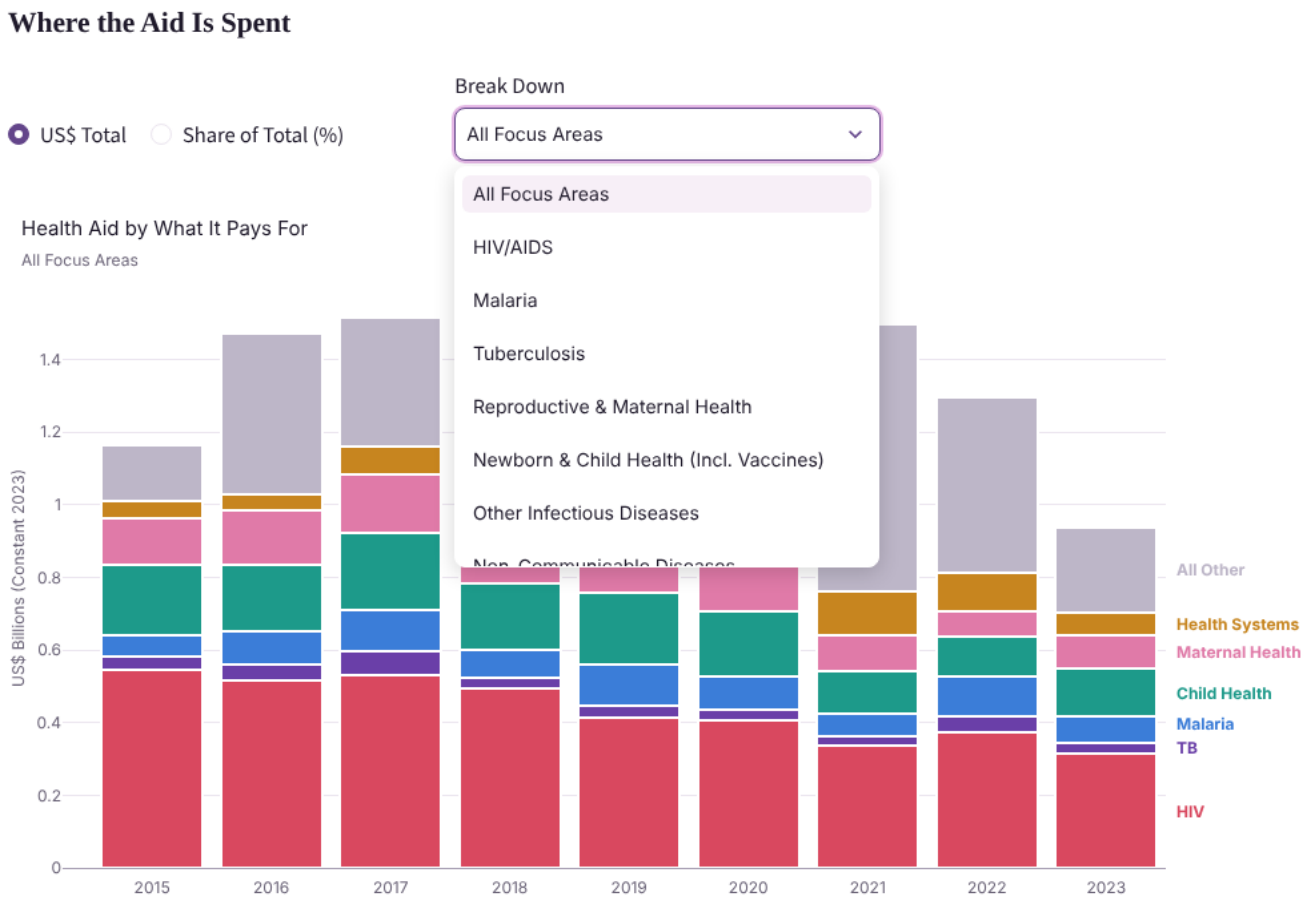
$$\mathrm{DAH}_c = \sum_s \sum_k A_{c,s,k}$$

$$\nu_c = \frac{1}{\mathrm{DAH}_c} \sum_s \sum_{k \in \mathrm{NGO}} A_{c,s,k}$$

$A_{c,s,k}$	aid from donor s through channel k
s	donor: US, UK, Gates, Germany, ...
k	channel: bilateral agency, NGO, Global Fund, Gavi, UN, ...
ν_c	share delivered through NGOs and foundations

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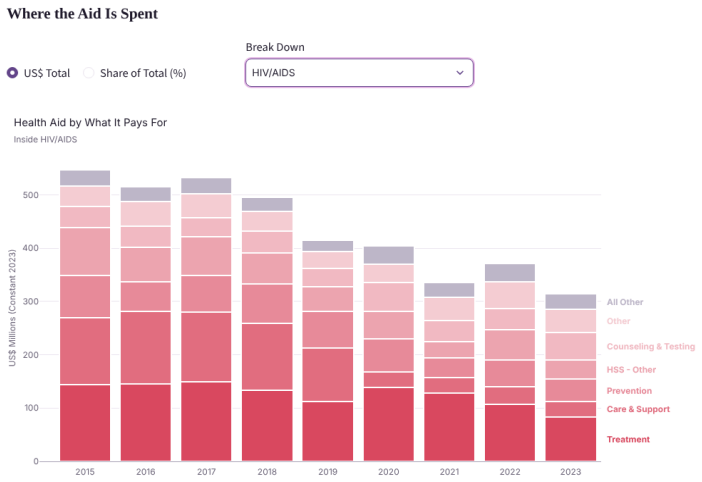
Dashboard – Resolving what program areas are funded by development assistance for each focus area



$$\mathrm{DAH}_c = \sum_d \sum_p A_{c,d,p}$$

$$A_{\mathrm{HIV}} = A_{\mathrm{treat}} + A_{\mathrm{PMTCT}} + A_{\mathrm{prev}} + A_{\mathrm{care}} + A_{\mathrm{test}} + A_{\mathrm{OVC}} + A_{\mathrm{HSS}} + A_{\mathrm{other}}$$

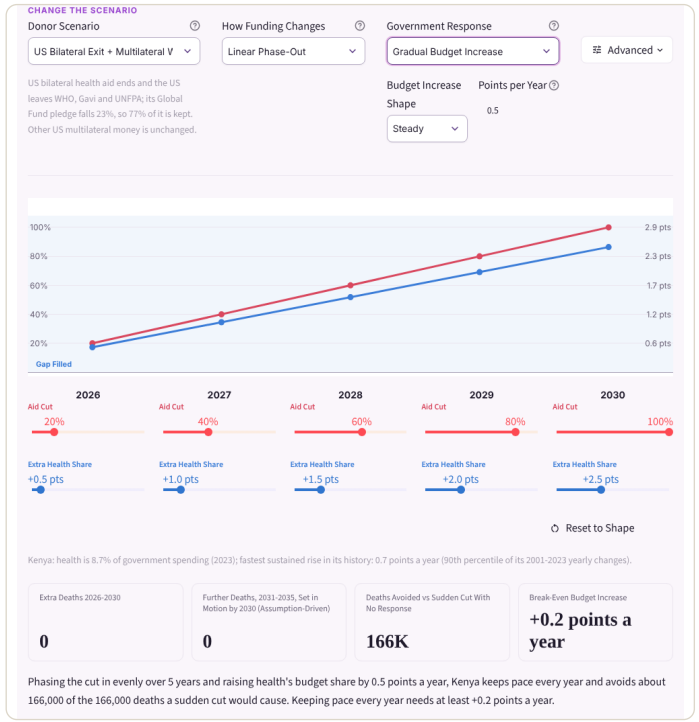
d	focus area: HIV, TB, malaria, child health, ...
p	program area within d
$A_{c,d,p}$	aid to program p of focus area d



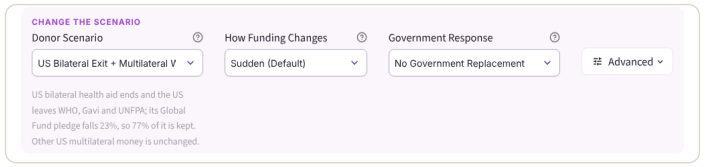
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Dashboard – Modeling the aid retreat

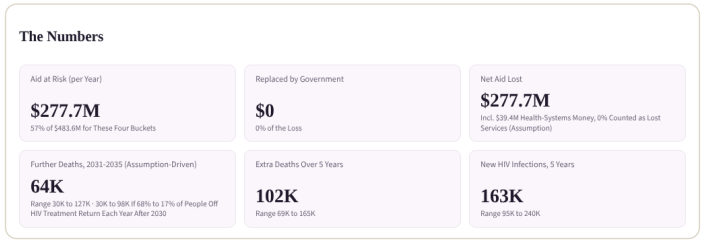
Funding path, year by year



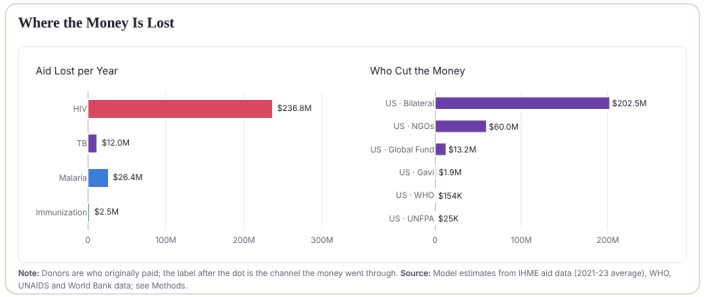
Scenario choices



Aid at risk



Where the money is lost



$$\kappa_{s,k} = 1 - (1 - x_s)(1 - y_k)$$

$$L_t = p_t \sum_{s,k,l} \bar{A}_{s,k,l}\, \kappa_{s,k}$$

$$\bar{A}_{s,k,l} = \frac{1}{3} \sum_{\tau=2021}^{2023} A_{s,k,l,\tau}$$

x_s	cut by donor s (set by the scenario)
y_k	cut through channel k
$\kappa_{s,k}$	combined cut on cell (s, k)
$\bar{A}_{s,k,l}$	2021–23 average aid on service line l
$p_t \in [0, 1]$	share of the cut in force in year t
L_t	aid at risk in year t

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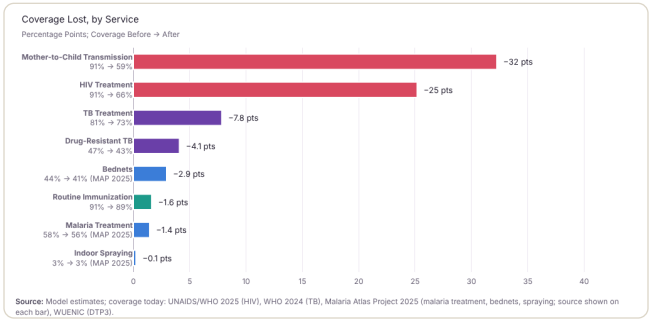
Dashboard – Modeling health coverage loss

What each service loses

What Each Service Loses

Service	Donor Aid Now per year	Aid Lost per year, after government replacement	Cost per Person	People Losing Service, per Year	Coverage	Extra Deaths, 5 Years Total
HIV Treatment antiretroviral therapy (ART)	$129.5M	$103.1M	$308 per patient-year of ART	378K People who stop ART: full-cycle equivalents of 42K to 100K	91% → 66% −25 pts	71K 42K to 100K
HIV Testing & Care counselling, testing, care & support	$77.6M	$67.6M	–	–	–	In HIV Treatment
Mother-to-Child Transmission prevention (PMTCT)	$8.1M	$5.0M	$250 per woman per year, infections over 5 years	14K 14K over 5 years	91% → 59% −32 pts	7.1K 3.3K to 11K
HIV Prevention PrEP, VMMC, condoms, key populations	$54.9M	$27.3M	$8K per infection averted	2.8K 14K over 5 years	No Single Target Group	189 71 to 311
TB Treatment case finding & treatment	$13.3M	$2.9M	$273 per patient	9.2K 46K over 5 years	81% → 73% −7.8 pts	20K 12K to 27K
Drug-Resistant TB second-line treatment	$1.3M	$239K	$5K per patient	40 198 over 5 years	47% → 43% −4.1 pts	92 54 to 124
Bednets insecticide-treated nets & other vector control	$31.6M	$10.8M	$2.20 per person-year	1.66M 8.32M over 5 years	44% → 41% (MAP 2025) −2.9 pts	1.2K 835 to 1.7K
Indoor Spraying indoor residual spraying	$7.7M	$1.8M	$8.50 per person-year	70K 350K over 5 years	3% → 3% (MAP 2025) −0.1 pts	32 21 to 43
Malaria Treatment testing & treatment	$21.9M	$5.2M	$14 per case	56K 278K over 5 years	58% → 56% (MAP 2025) −1.4 pts	1.6K 1.1K to 1.9K
Routine Immunization vaccines + delivery	$27.5M	$2.5M	$87 per child	25K 124K over 5 years	91% → 89% −1.6 pts	966 748 to 1.9K
Other Aid orphan support; systems money not cutting services	$110.3M	$51.2M		Not Modelled		Not Modelled
Total	$483.6M	$277.7M		2.22M		102K

Coverage lost, by service



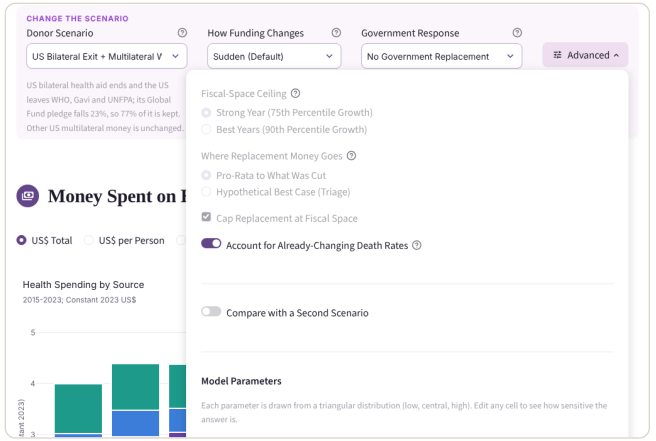
$$\Delta U_{l,t} = (1-\rho)\,\min\left\{\frac{N_{l,t}}{u_l},\ \delta_l\, C_l\, P_l\right\}$$

$$\Delta C_{l,t} = \Delta U_{l,t} \,/\, P_l$$

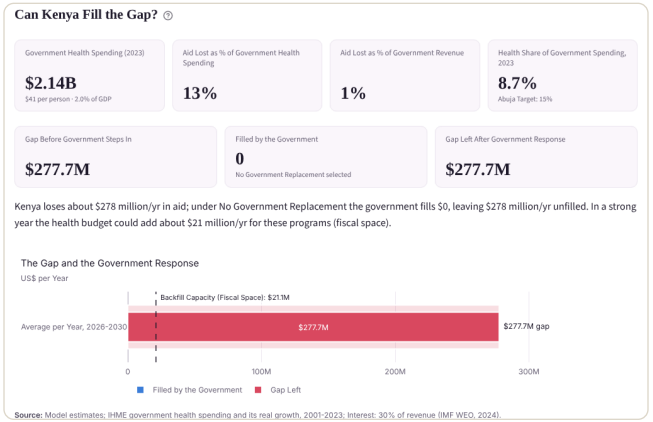
$N_{l,t}$	net aid lost on line l, after step 3
u_l	full cost per person served
P_l	people in need
C_l	coverage today
δ_l	donor share of spending on the disease
ρ	share who keep care another way (0.15)
$\Delta U_{l,t}$	people losing the service
$\Delta C_{l,t}$	coverage drop (simplified rule)

Dashboard – Modeling how fiscal space within country budgets can be used to buffer cuts

Response settings



Can the country fill the gap?



$$K = \mathrm{GHES}_{2023}\left(g^{p75} - g^{\mathrm{med}}\right)\sigma(i)$$

$$\sigma(i) = \min\left\{\max\left[1 - \frac{i-10}{40},\ 0.25\right],\ 1\right\}$$

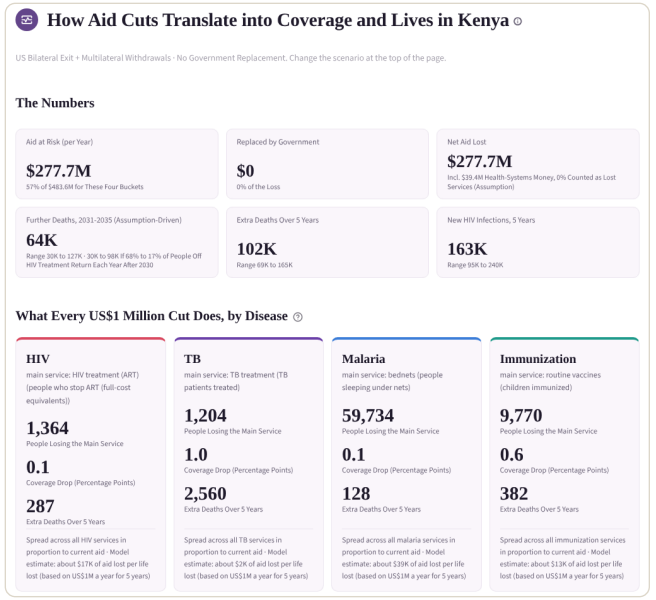
$$R_t = \min\{\theta L_t,\ K\}$$

$$N_t = L_t - R_t$$

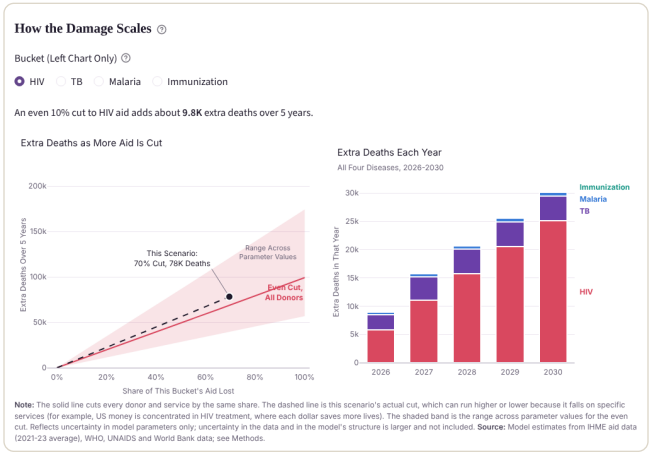
K	fiscal space for health, per year
$g^{p75},\ g^{\mathrm{med}}$	strong-year and median real growth in GHES, 2001–23
i	interest as % of revenue
$\sigma(i)$	debt-stress factor
θ	share of the loss the government chooses to replace
R_t	aid replaced by government
N_t	net loss, passed to step 2

Dashboard – Modeling lives at risk as a result of cuts

Deaths and US$1 million cards



How the damage scales



$$D = \sum_{l} \sum_{t=2026}^{2030} \Delta U_{l,t}\, m_l(\tau)$$

$$m_{\mathrm{ART}}(\tau) = \mu\, h_\tau, \qquad h = (0.013, \ldots, 0.060)$$

$$m_{\mathrm{TB}} = \mathrm{CFR}^{u} - \mathrm{CFR}^{tx}$$

$$D_1^{\mathrm{mal}} = D_0^{\mathrm{mal}} \frac{1 - E_v C_{v,1}}{1 - E_v C_{v,0}} \cdot \frac{1 - E_c C_{c,1}}{1 - E_c C_{c,0}}$$

$m_l(\tau)$	deaths per person losing l, au years after
h_τ	excess death rate off ART (Optima HIV)
μ	uncertainty multiplier
CFR^{u}, CFR^{tx}	TB case fatality, untreated and treated
E_v, E_c	effect of vector control, case management
C_v, C_c	their coverage, before (0) and after (1)

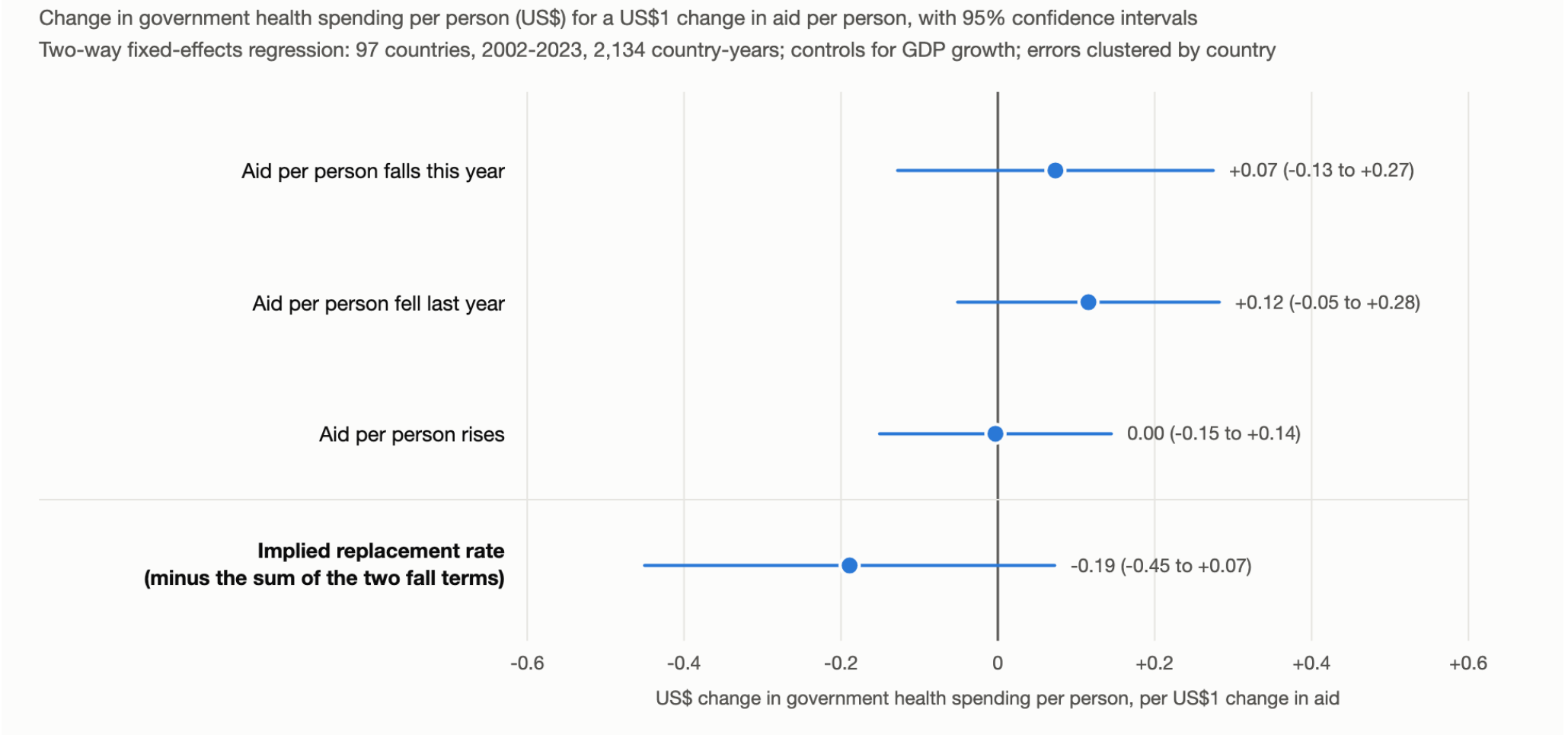
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Live Demo: Choosing A Country And Scenario Reveals Which Services An Aid Cut Puts At Risk

Open The Live Dashboard ›

1. **Pick A Country And Scenario**
2. **See The Aid At Risk**
3. **Set The Government Response**
4. **Count People Losing Services**
5. **Read The Coverage Change**

Historical patterns show that when the US decreases aid, countries decreased domestic government health expenditure by 19 cents

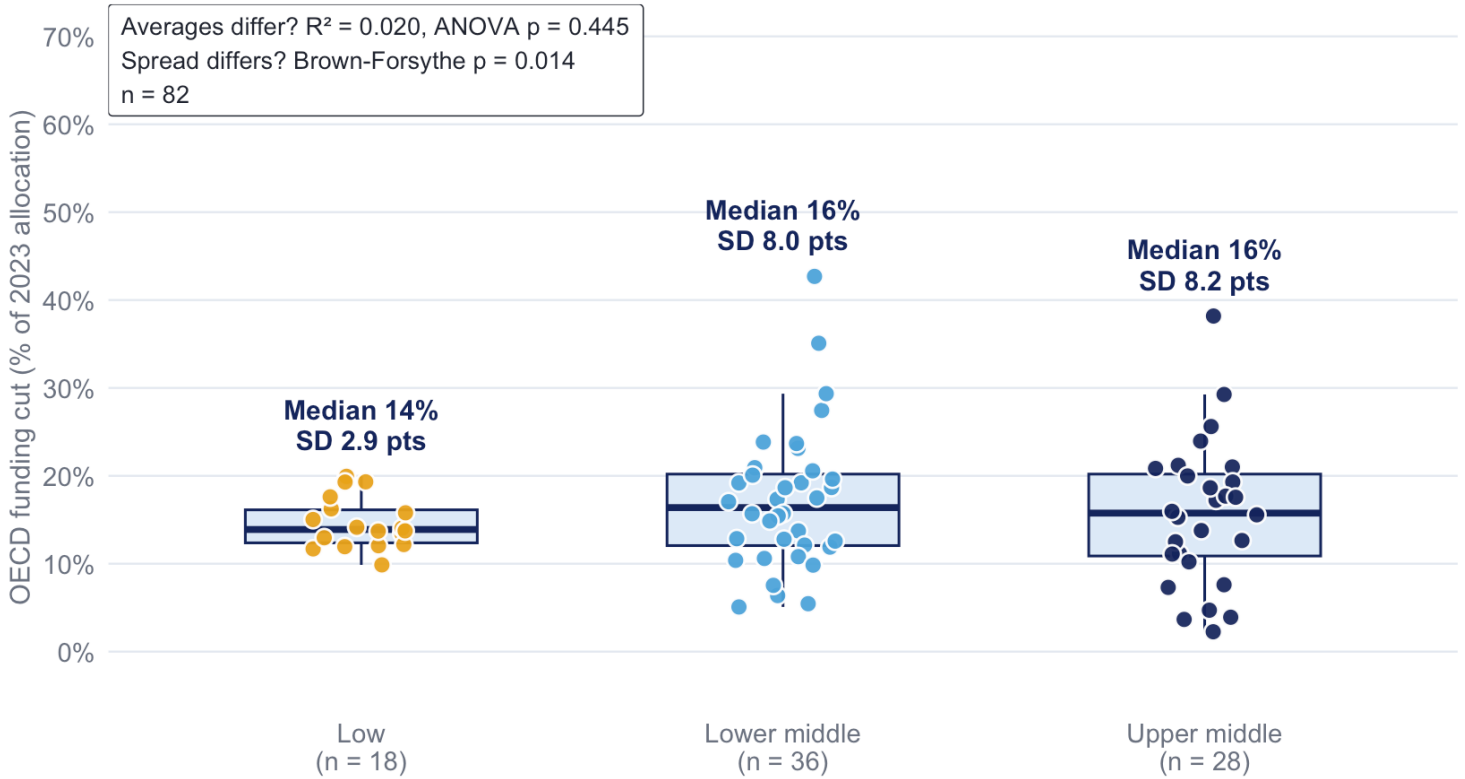


04
Findings

04.1

Domestic resource mobilization capacity – can countries grow their way out of the problem?

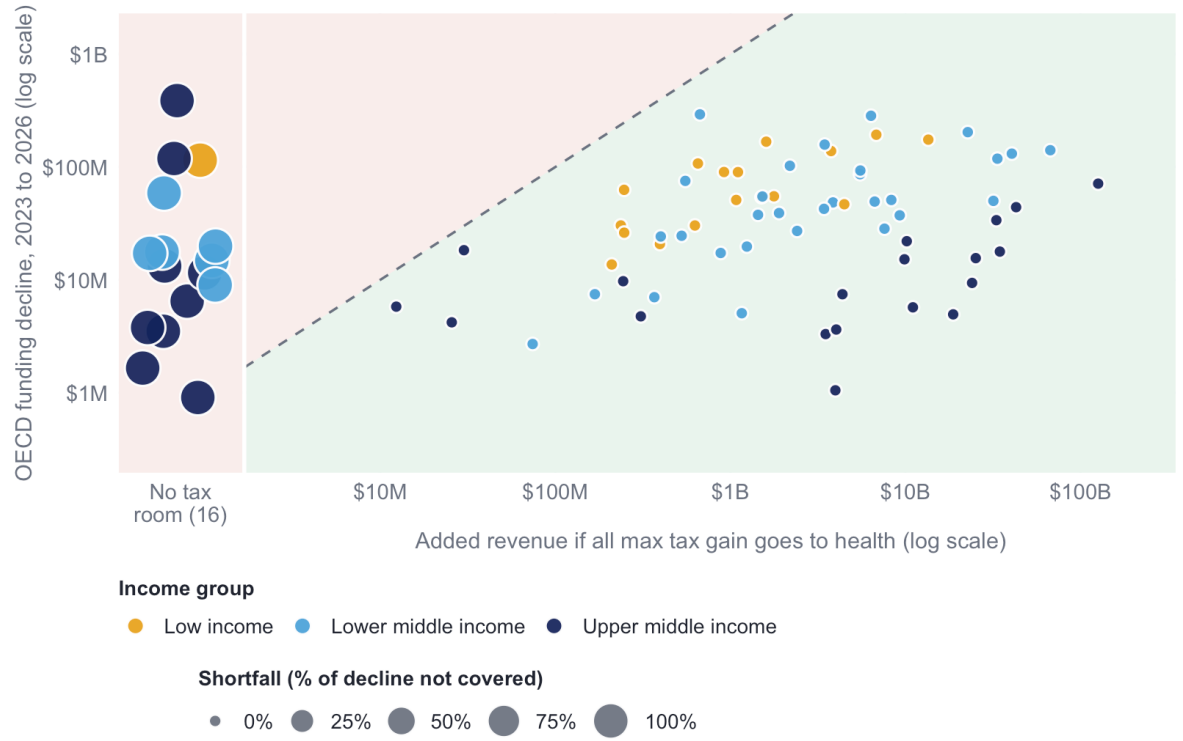
Typical OECD cut averages were similar across income groups



Sources: Income classification from the World Bank. OECD cuts are projections based on OECD data and official country-level communications.

Typical cuts were similar across income groups, at 14% to 16% of 2023 allocations. The spread was not: every low-income country lost 10% to 20%, while middle-income countries ranged from 2% to 43%.

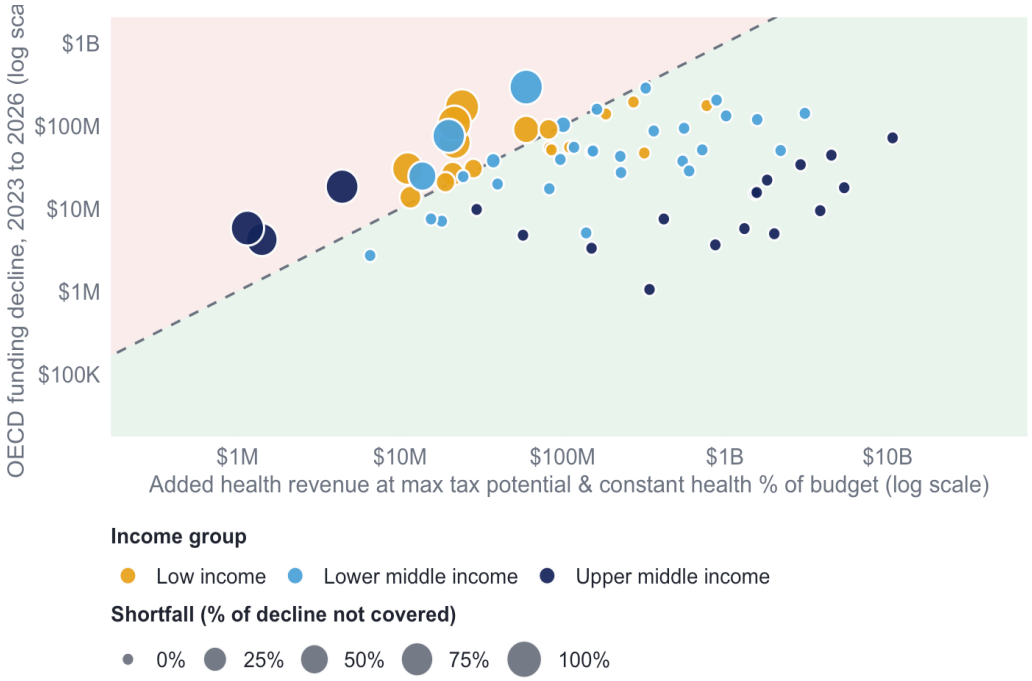
All countries that have tax potential to grow could generate enough revenue to cover the gap

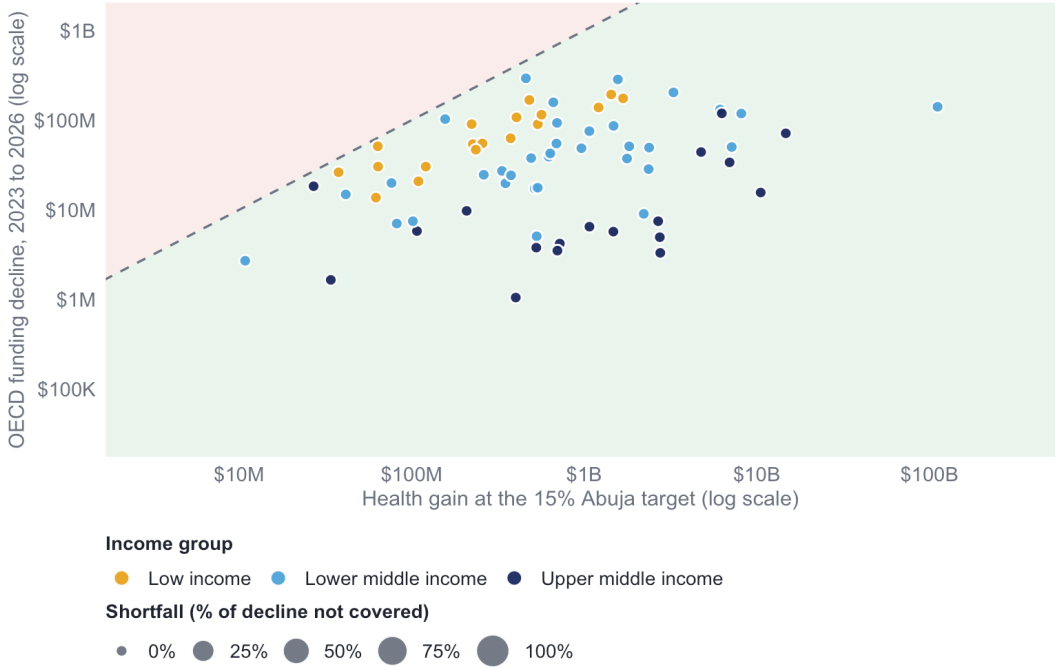


Sources: Tax revenue set to IMF-estimated tax potential (Benitez et al., 2023); OECD funding cuts are projected using OECD cuts are projections based on OECD data and official country-level communications.

Assuming all new funds were allocated to health, revenue potential is not the constraint for the potential for countries to cover the gap. However, it is unlikely that countries would allocate all new funds to health.

Revenue growth is enough for most, but not for a critical few who additionally require budgetary changes

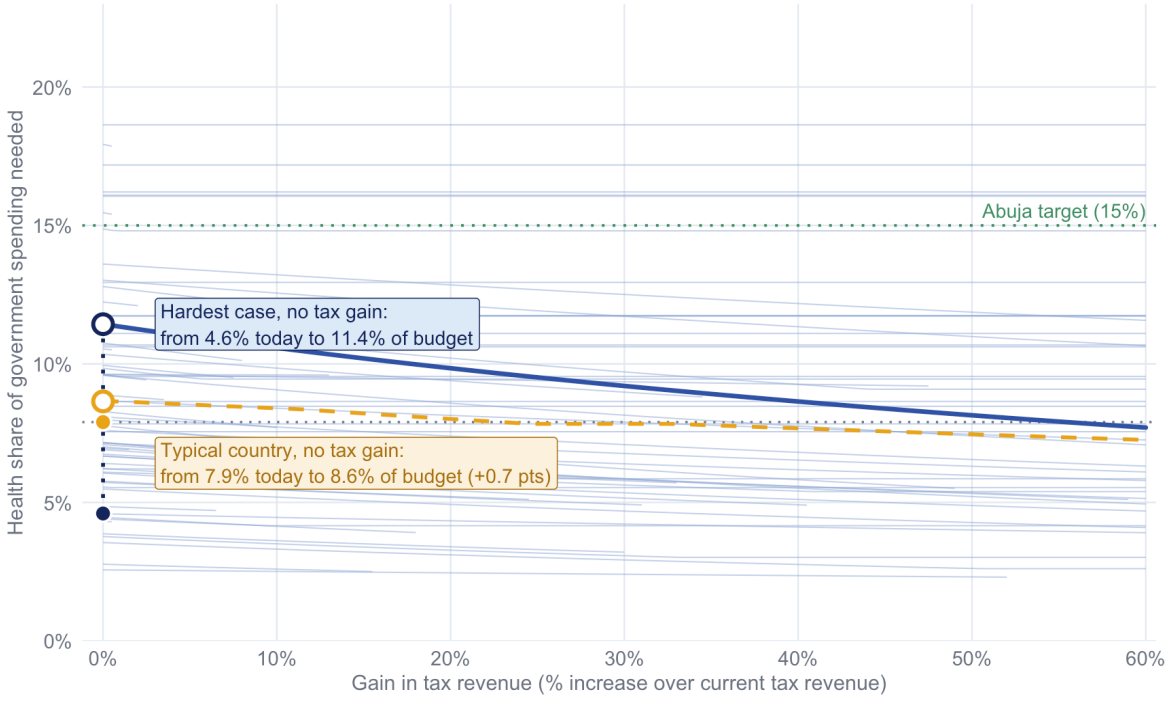


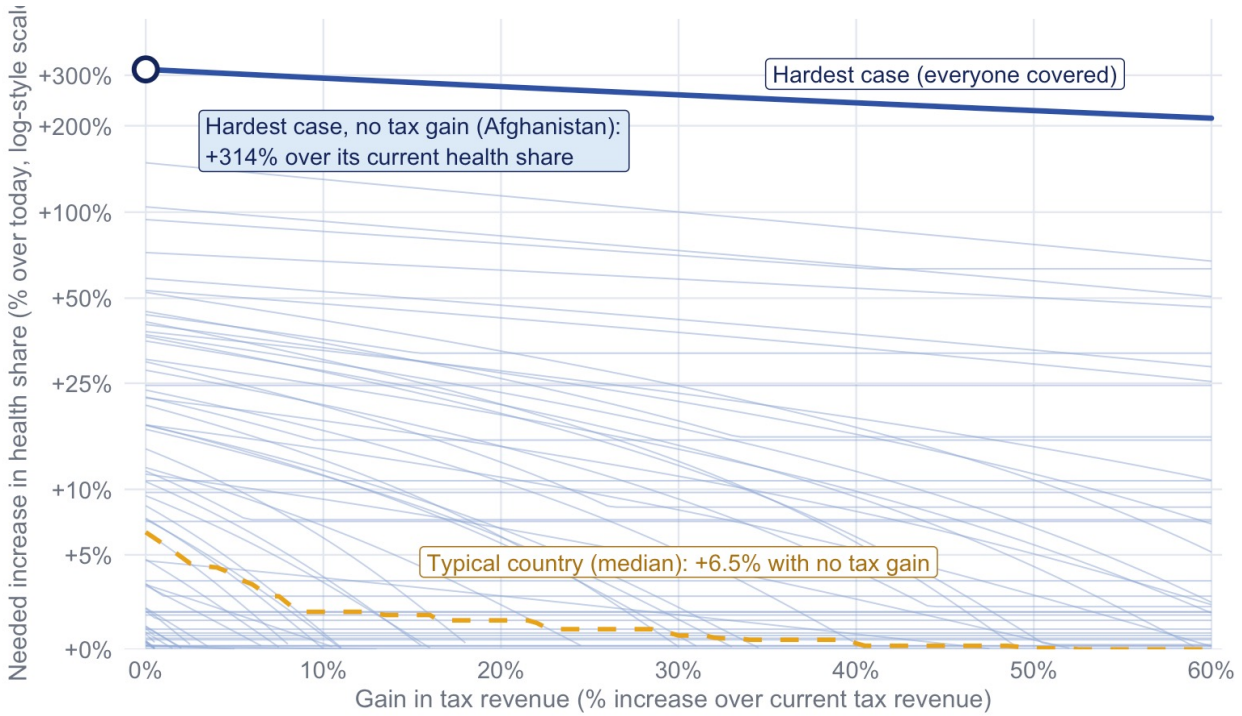


Sources: Max tax gain = IMF-estimated tax potential (Benitez et al., 2023) minus current tax revenue (IMF), capped at 9% of GDP. Health gain assumes the current health share of the government budget (World Bank) holds on new revenue. 16 countries already at or above their tax potential are not shown. OECD cuts are projections based on OECD data and official country-level communications.

Potential tax revenue growth still covers the decline in 48 of 82 countries if health expenditure as a % of budget is kept constant. Countries with large cuts and low health expenditure % of overall expenditure would need revenue growth beyond what is likely realistic to close the gap through taxes alone.

Most countries need only a modest shift in budget priority, but a few would need to at least double their health share





Sources: Health expenditure as a % of government budget is sourced from the World Bank and current tax revenue is sourced from the IMF. OECD cuts are projections based on OECD data and official country-level communications.

Budget reallocation in absolute terms may be a small shift for the typical country (+0.7 points), but 21 countries start below 5% of their budget and would have to raise health's share sharply.

Loans tied to health priorities can incentivize new funds to be invested into health

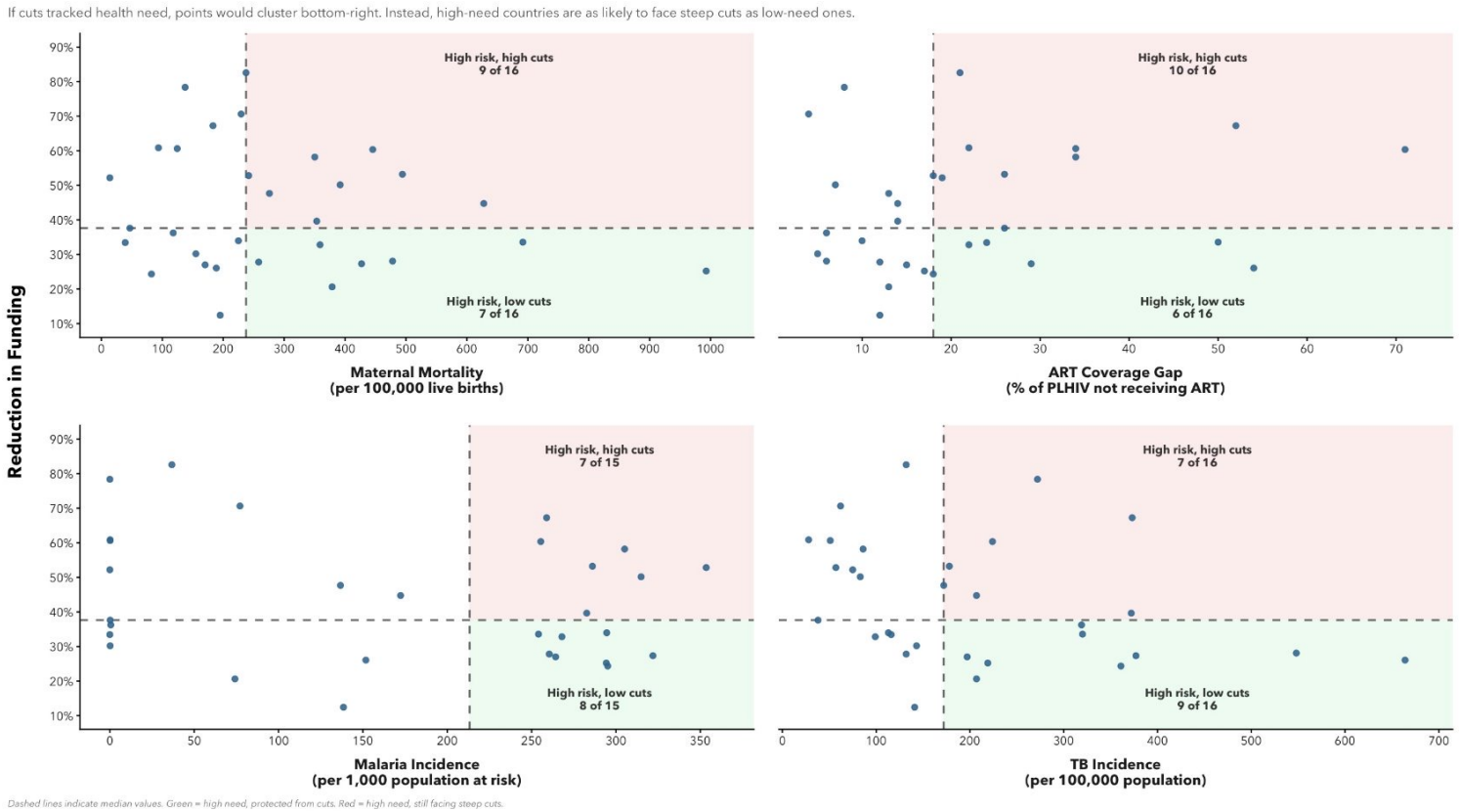
Strategy	Description
Sustainability-Linked Loans	Interest rate or repayment schedule improves when national health targets are met
Matching Domestic Mandates	Low-interest loans are distributed when health investment increases

New revenue on its own competes with other budgetary demands, however, loans tied to health targets provide a potential avenue for circumventing this

04.2

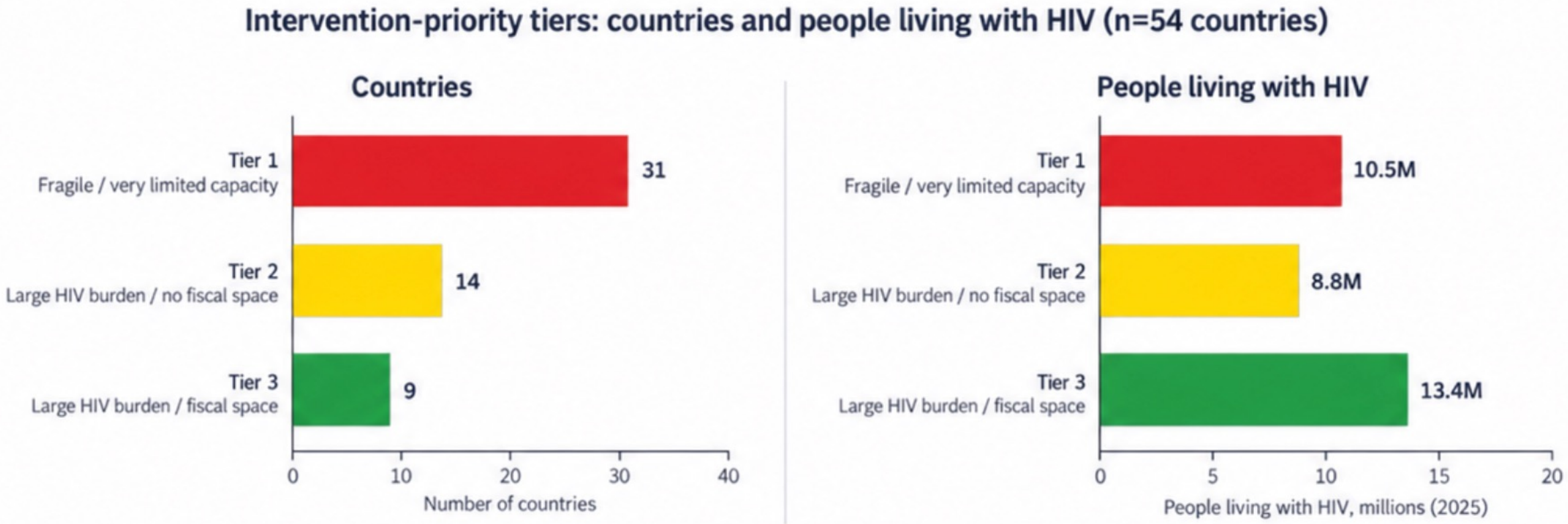
DAH prioritization – where should donors be targeting remaining funds?

Recall: Funding cuts show no consistent relationship with health need, leaving high risk countries vulnerable



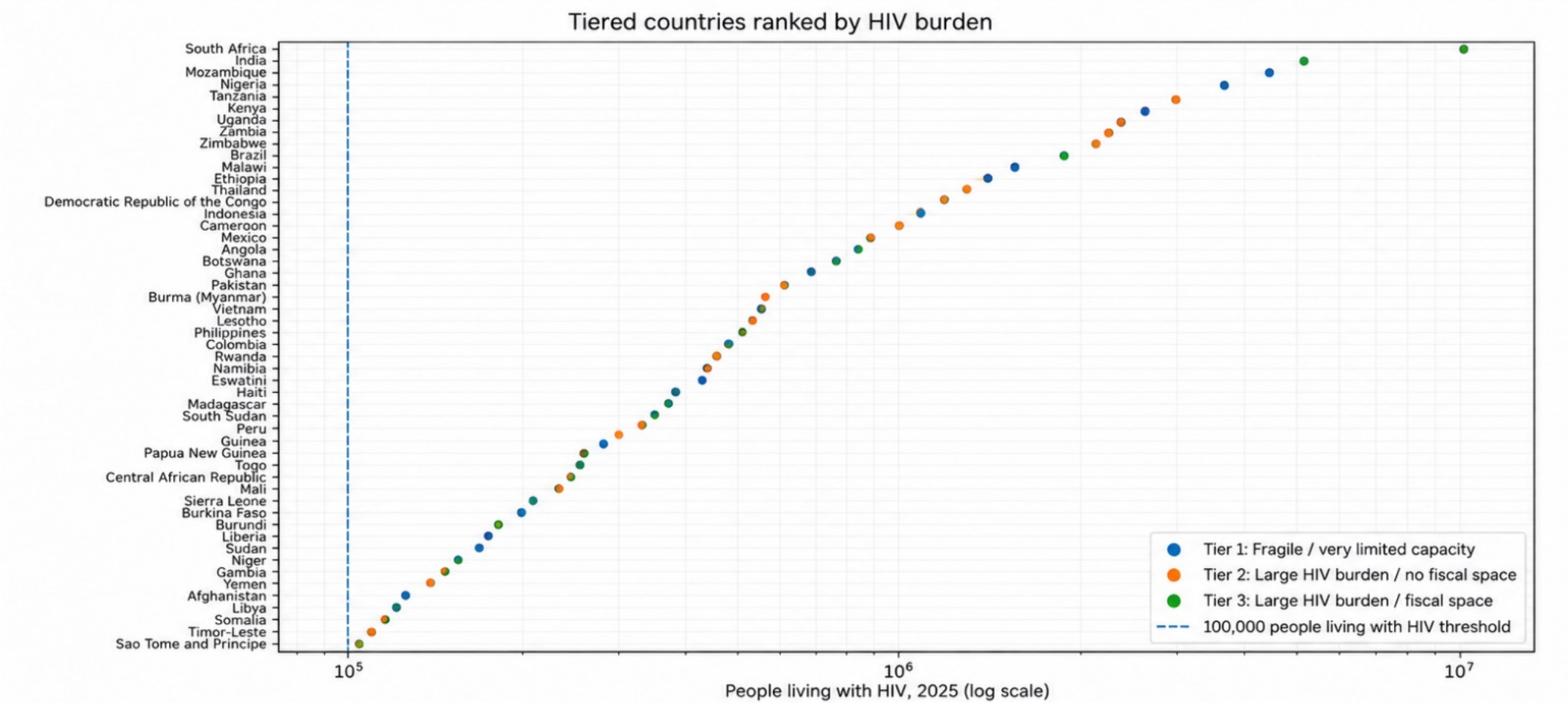
Cuts also did not follow health need, leaving some high-risk countries heavily exposed and others spared. Uneven exposure requires tailored responses

HIV Financing Transition: Country Risk-Tier Analysis



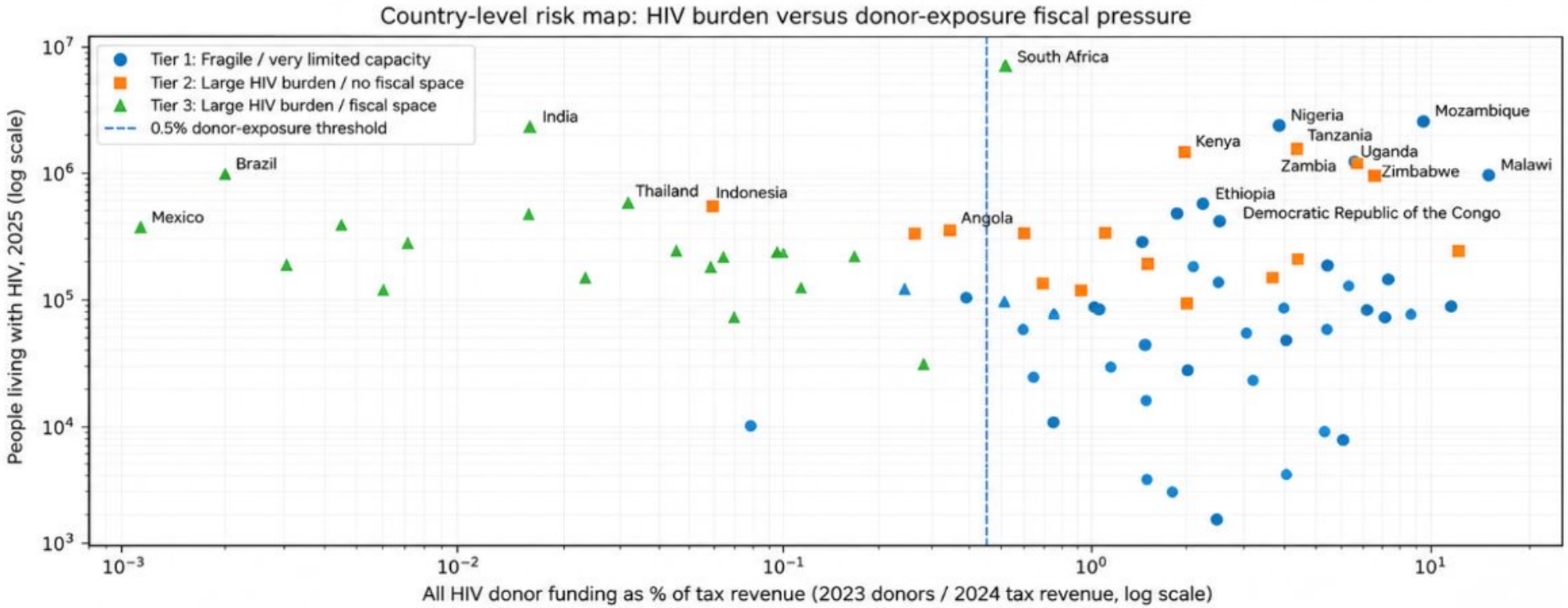
Total people living with HIV across tiered countries: 32.7 million.

Country risk map



1. Operational large-burden test = people living with HIV ≥100,000 or people living with HIV ≥1% of population.
2. All 51 tiered countries with available 2025 HIV estimates are plotted. Iraq, Ukraine, and West Bank and Gaza are tiered in the workbook but are not plotted because 2025 HIV estimates are missing.

Countries by HIV burden



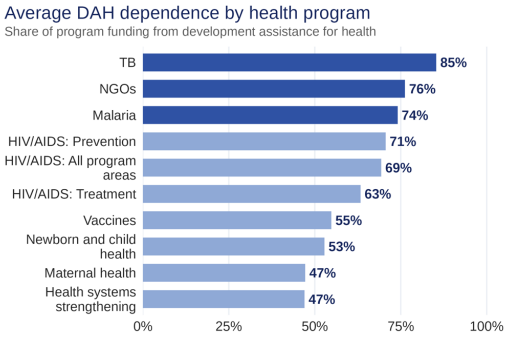
1. Fiscal space is screened using two indicators: (1) all HIV donor funding as a share of tax revenue and (2) public and publicly guaranteed external debt service as a share of government revenue.
2. Constrained fiscal space = donor exposure ≥0.5% of tax revenue and/or public and publicly guaranteed external debt service ≥15% of government revenue.
3. The x-axis visualizes the donor-exposure indicator only; the debt-service indicator is also used in tier assignment.
4. All 51 countries with available HIV estimates are plotted; Iraq, Ukraine, and West Bank and Gaza are tiered in the workbook but are not plotted because 2025 HIV estimates are missing.
5. Fragile / very limited capacity refers to conflict / institutional fragility markers and severe implementation constraints; it is distinct from the scale of HIV burden.

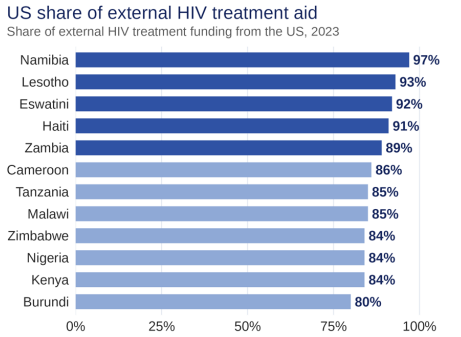
Priority tiers and intervention emphasis

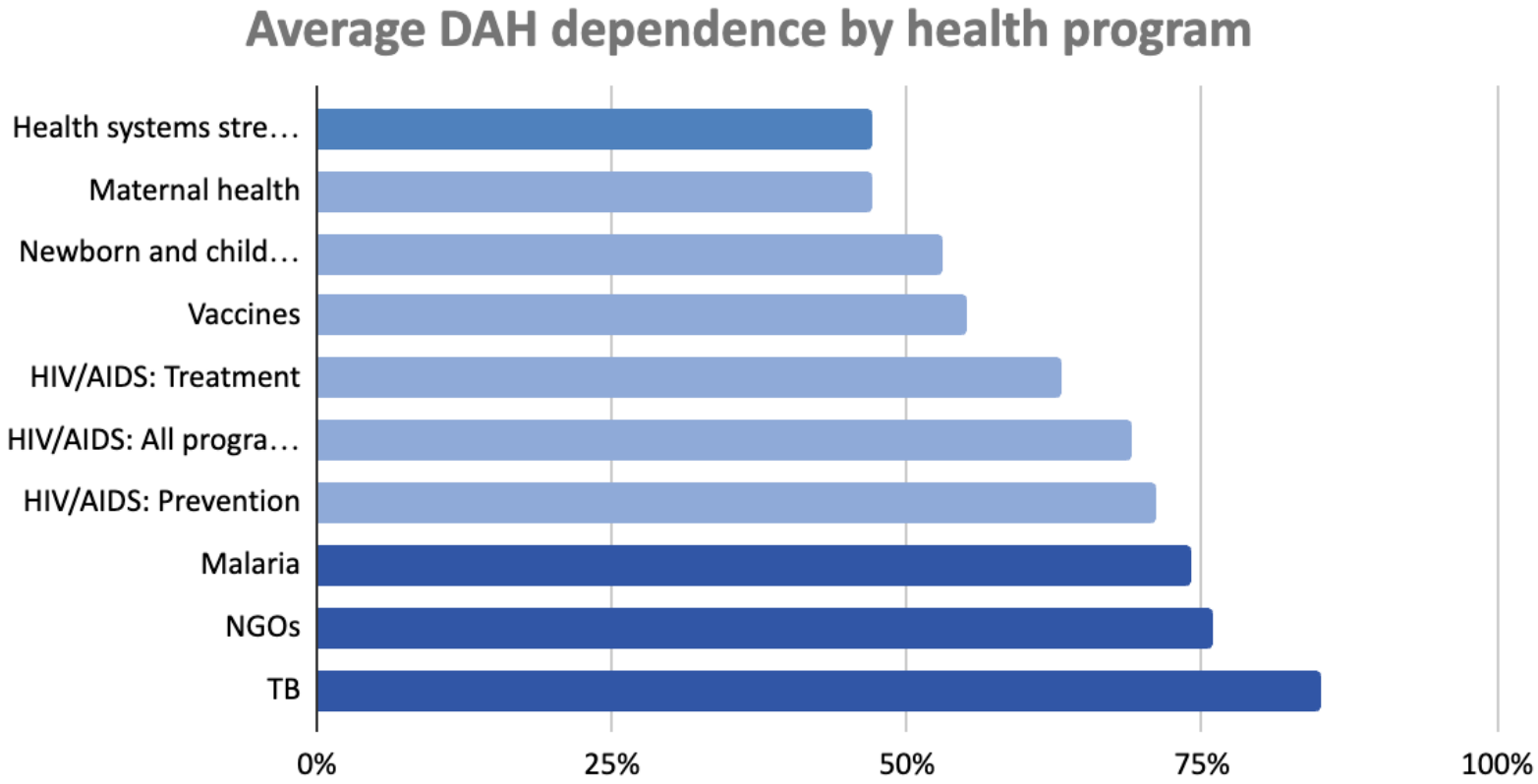
Tier	Working definition	Strategic intervention emphasis
Tier 1: Fragile / very limited capacity	Fragile or very limited-capacity settings. Priority is triggered by conflict / institutional fragility markers and severe implementation constraints. Countries in this tier may or may not have a large HIV burden.	Protect continuity of core HIV services, maintain supply chains and community delivery, and prioritize bridge financing.
Tier 2: Large HIV burden / no fiscal space	Large HIV burden and constrained fiscal space. Large HIV burden = people living with HIV ≥100,000 or people living with HIV ≥1% of population. Constrained fiscal space = all HIV donor funding ≥0.5% of tax revenue and/or public and publicly guaranteed external debt service ≥15% of government revenue.	Protect high-volume services and negotiate realistic domestic co-financing, phased transition, and efficiency measures without abrupt funding cliffs.
Tier 3: Large HIV burden / fiscal space	Large HIV burden but relatively greater fiscal space. These countries meet the large-burden test but remain below the fiscal-pressure thresholds and are better positioned for planned domestic absorption over time.	Prioritize planned domestic absorption, medium-term budget commitments, and targeted external support for gaps that domestic systems cannot immediately absorb.

Fragility and large HIV burden are distinct concepts: fragility describes state capacity and implementation constraints; large HIV burden describes the scale of the epidemic. Fiscal space is screened using tax revenue and debt-service pressure.

High-dependency programs are where remaining donor funds matter most







UNAIDS) 2026 Estimates

donor response

05

Looking Ahead

Calendar

Event	Date
Current Meeting	October 13, 2026
Final Readout	TBD
Report Finalized	TBD
Final Presentation Competition	December 14, 2026

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Next Steps and Discussion

- Estimate how long it might take for fiscal space to grow
- Model various concessional loan terms and their impact
- Work on the written report and final presentation
- Anything else?